

HOSPITAL DISCHARGE SUMMARY

PATIENT INFORMATION:

Name: Patient 04

Age: 81 years old

Sex: Female

MRN: 000-TEST-04

Admission Date: [admission date]

Discharge Date: [discharge date]

Length of Stay: [LOS] days

Attending Physician: [Provider Name], MD

PRIMARY DIAGNOSIS:

Congestive Heart Failure with reduced ejection fraction (HFrEF), NYHA Class III-IV

SECONDARY DIAGNOSES:

- Vascular dementia with behavioral disturbance
- Osteoporosis s/p prior vertebral compression fracture
- Hypertension
- Chronic kidney disease Stage III (eGFR 42 mL/min/1.73m²)
- Anemia of chronic disease
- Hypothyroidism

HOSPITAL COURSE:

Patient 04 presented to Metro Medical Center with acute decompensated heart failure characterized by orthopnea, paroxysmal nocturnal dyspnea, and lower extremity edema. Initial labs revealed significantly elevated B-type natriuretic peptide (BNP) of 1,247 pg/mL. Transthoracic echocardiogram demonstrated severe systolic dysfunction with ejection fraction (EF) of 22%, global hypokinesis, and moderate mitral regurgitation. Patient was admitted to telemetry monitored bed for diuretic optimization and ACE inhibitor titration.

Hospital course notable for:

- IV furosemide drip initiated, transitioned to oral diuretics
- Daily weights obtained; net diuresis of 4.2 kg achieved over admission
- Lisinopril uptitrated from 5 mg q.d. to 10 mg q.d.
- Carvedilol initiated and titrated to 6.25 mg BID (goal-directed medical therapy per HF guidelines)
- Mild hyperkalemia (K⁺ 5.8) managed conservatively; repeat labs normal
- Cognitive status remained stable; no acute delirium. Dementia-related behavior managed with non-pharmacologic interventions
- Discharged to home with family caregiver support

DISCHARGE MEDICATIONS:

Medication	Dose	Frequency	Duration
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Furosemide (Lasix)	40 mg	BID	Ongoing
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Lisinopril	10 mg	q.d.	Ongoing
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Carvedilol	6.25 mg	BID	Ongoing
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Levothyroxine	50 mcg	q.d.	Ongoing
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Calcium carbonate + Vitamin D	500 mg + 400 IU	BID with meals	Ongoing
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Aspirin	81 mg	q.d.	Ongoing
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Atorvastatin	20 mg	q.d.	Ongoing
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Docusate (Colace)	100 mg	BID	As needed
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ACTIVITY & WEIGHT-BEARING STATUS:

Activity as tolerated. Avoid heavy lifting (>5 lbs). Use assistive devices when ambulating due to osteoporosis and fall risk. Encourage seated activities. NO stair climbing without supervision.

DIETARY RESTRICTIONS:

STRICT SODIUM RESTRICTION: <2 grams (2,000 mg) per day

FLUID RESTRICTION: 1.5 liters per 24 hours (including all beverages and ice chips)

Diabetic diet NOT required; however, heart-healthy, low-sodium meals encouraged

MONITORING INSTRUCTIONS:

Daily weights each morning before breakfast. Report weight gain >2-3 lbs in one day or >5 lbs in one week. Check for peripheral edema daily. Monitor orthostasis and dizziness